**1. A parent brings in their 3-year-old child who just recovered from a 4-day bout of extremely high fever (104°F). As the fever subsided, a maculopapular rash appeared on the trunk. You observe small red dots on the soft palate and uvula. What are these dots called?**

A. Koplik spots

B. Forchheimer’s spots

C. Nagayama spots

D. Pastia’s lines

**2. A 20-year-old college student presents with extreme fatigue, a sore throat, and "petechiae on the soft palate." Clinical examination reveals bilateral tonsillar enlargement with exudate. Which test is used to detect heterophile antibodies for this condition?**

A. CBC with differential

B. Monospot Test

C. PCR for HHV-1

D. TPHA Assay

**3. A patient presents with painful "crops of vesicles" specifically on the attached gingiva and hard palate. They mention a burning sensation occurred 24 hours before the lesions appeared. This recurrence typically involves which nerve ganglion?**

A. Dorsal root ganglion

B. Geniculate ganglion

C. Trigeminal ganglion

D. Pterygopalatine ganglion

**4. A 6-year-old child presents with fever, rhinitis, and cough. You notice small whitish-gray spots on the buccal mucosa. What is the most likely diagnosis?**

A. Rubella

B. Roseola

C. Rubeola

D. Herpangina

**5. A child presents with a skin rash on their palms and soles. Intraorally, you observe red macules that have formed vesicles and ulcers on the tongue and labial mucosa. Which virus is the most likely causative agent?**

A. Rubulavirus

B. Morbillivirus

C. Enteroviruses

D. Togavirus

**6. A male patient presents with significant bilateral swelling below the jawline and reports recent testicular pain. What is the specific name for this condition?**

A. Epidermic Parotitis

B. Cervical Lymphadenitis

C. Scrofula

D. Herpetic Whitlow

**7. An HIV+ patient presents with vertical white plaques on the lateral border of the tongue. This clinical presentation of Oral Hairy Leukoplakia is caused by:**

A. HHV-5 (CMV)

B. HHV-4 (EBV)

C. HHV-8 (KSHV)

D. Human Papilloma Virus (HPV)

**8. A 5-year-old patient has small red lesions (1-2 mm) located specifically on the tonsillar pillars in the posterior pharynx. They have flu-like symptoms but no skin rash. This is likely:**

A. Hand-Foot-Mouth Disease

B. Herpangina

C. Bacterial Pharyngitis

D. Scarlet Fever

**9. A patient presents with a painful, unilateral vesicular eruption following a specific dermatome on the chest. They mention having "Chicken Pox" as a child. Where did this virus remain latent?**

A. Trigeminal ganglion

B. CD4+ T cells

C. Dorsal root ganglion

D. Salivary glands

**10. A patient presents with multiple "warty lesions" on the oral mucosa. You are concerned about malignant transformation. Which type of HPV is specifically mentioned as a high-risk type linked to malignancy?**

A. HPV 6

B. HPV 11

C. HPV 16

D. HPV 42

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## Pregnancy & Dental Management Cases

**11. A patient in her 18th week of pregnancy presents with a smooth, vascular mass on the gingiva that bleeds easily when she brushes. What is the most likely diagnosis?**

A. Pregnancy Gingivitis

B. Pyogenic Granuloma

C. Epulis Fissuratum

D. Condyloma Acuminatum

**12. The same patient mentions she is afraid of "gum disease" causing her baby to be born early. According to the theoretical "Ascending Infection" pathway, what mediator leads to uterus contraction?**

A. C-reactive protein

B. Prostaglandin E2 (PGE2)

C. Interleukin-1

D. TNF-alpha

**13. A pregnant patient is suffering from severe morning sickness. You advise her to rinse with a specific solution after vomiting instead of brushing immediately. What should the rinse contain?**

A. Chlorhexidine

B. Hydrogen peroxide

C. Baking soda and water

D. Fluoride and alcohol

**14. You need to perform an emergency root canal on a pregnant patient. Which trimester is considered the optimal time for this dental care?**

A. First Trimester

/B. Second Trimester

C. Third Trimester

D. Postpartum

**15. You need to prescribe an antibiotic to a pregnant patient with an odontogenic infection. According to the FDA risk categories, which of the following is a Category B antibiotic?**

A. Tetracycline

B. Doxycycline

C. Penicillin

D. Ciprofloxacin

**16. Which anesthetic mentioned in the lectures should be specifically avoided in the first trimester because it may result in "neonatal depression and spontaneous abortion"?**

A. Lidocaine

B. Prilocaine

C. Nitrous Oxide

D. Mepivacaine

**17. A pregnant patient asks why her gums are more swollen now than before pregnancy. You explain that a specific bacteria uses her fluctuating hormones (steroids) as growth factors. Which bacteria is this?**

A. Streptococcus mutans

B. Porphyromonas gingivalis

C. Prevotella intermedia

D. Fusobacterium nucleatum

**18. When educating a new mother, at what age does the lecture recommend a child's first dental visit should occur?**

A. 6 months

B. 1 year

C. 2 years

D. 3 years

**19. A patient is in her third trimester. Which virus, if contracted now, is associated with up to 30% mortality in pregnant females due to fulminant hepatitis?**

A. Hepatitis A

B. Hepatitis B

C. Hepatitis C

D. Hepatitis E

**20. According to the "Lifelong Continuum," maternal periodontitis is linked to which fetal development risk?**

A. Cleft palate

B. Enamel hypoplasia

C. Macroglossia

D. Delayed eruption

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## Bacterial Pathogenesis & STIs

**21. A patient presents with "punched-out" interdental papillae, red, necrotic gingiva, and a strong "fetid" odor. What is the common name for this condition?**

A. Strawberry Tongue

B. Trench Mouth

C. Scrofula

D. Luetic Glossitis

**22. A patient presents with a painless ulcer (chancre) on the tongue. They mention it appeared about a month after a sexual encounter. Which organism is responsible?**

A. Bartonella henselae

B. Mycobacterium tuberculosis

C. Treponema pallidum

D. Neisseria gonorrhoeae

**23. If the patient in the previous question is not treated and returns years later with "palatal perforation" and indurated, ulcerated lesions, what are these lesions called?**

A. Mucous patches

B. Condyloma lata

C. Gummas

D. Split papules

**24. A child presents with chronic regional lymphadenopathy (swollen lymph nodes) in the neck and preauricular area. The parent mentions the child was recently scratched by the family cat. What is the causative agent?**

A. Bartonella henselae

B. Streptococcus pyogenes

C. Morbillivirus

D. Actinomyces israelii

**25. A patient presents with a productive cough, weight loss, and "chronic painless ulcers" in the mouth. A Ziehl-Neelsen stain of a sample shows red, rod-shaped bacilli. What is the diagnosis?**

A. Syphilis

B. Tuberculosis

C. Scrofula

D. Cat-Scratch Disease

**26. You observe a patient with notched incisors ("screwdriver teeth") and "mulberry molars." This "Hutchinson's Triad" is a sign of:**

A. Congenital Rubella

B. Congenital Syphilis

C. Primary Tuberculosis

D. Secondary Syphilis

**27. A patient presents with a sore throat, yellow exudate on the tonsils, and a skin rash described as "sunburn with goose pimples." What is the likely diagnosis?**

A. Scarlet Fever

B. Herpangina

C. Infectious Mononucleosis

D. Mumps

**28. Which bacterial toxin interferes with the release of GABA, leading to spastic paralysis and "lockjaw"?**

A. Botulinum toxin

B. Diphtheria toxin

C. Tetanospasmin

D. Shiga toxin

**29. A patient has been on broad-spectrum antibiotics for three weeks and now has severe diarrhea and abdominal pain. This is likely due to the over-proliferation of:**

A. Helicobacter pylori

B. Clostridium difficile

C. Pseudomonas aeruginosa

D. Staphylococcus aureus

**30. Which specific component of Gram-negative bacteria is the "endotoxin" that triggers fever and shock?**

A. Teichoic acid

B. Peptidoglycan

C. Lipid A (LPS)

D. M protein

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## Biofilms, Caries & Virology

**31. You are explaining plaque formation to a patient. Which type of attachment involves "weak, long-range, van der Waals forces" and occurs after the pellicle forms?**

A. Irreversible attachment

B. Reversible attachment

C. Co-aggregation

D. Biofilm maturation

**32. During a periodontal exam, you find 8mm pockets. Which complex of bacteria is found mostly in these deep pockets?**

A. Yellow complex

B. Green complex

C. Orange complex

D. Red complex

**33. Name the three bacteria that make up the "Red Complex" in periodontal disease.**

A. P. intermedia, F. nucleatum, P. micros

B. P. gingivalis, T. forsythia, T. denticola C. S. mutans, S. mitis, S. oralis D. A. viscosus,

A. naeslundii, L. casei

**34. In the shift from periodontal health to disease, the microbial composition changes from predominantly:**

A. Gram-negative to Gram-positive

B. Gram-positive facultative to Gram-negative anaerobic

C. Motile to non-motile D. Asaccharolytic to sugar-fermenting

**35. Which bacteria is the "primary initiator" of dental caries and uses glucosyltransferase to create sticky plaque?**

A. Lactobacillus casei

B. Streptococcus sanguis

C. Streptococcus mutans

D. Actinomyces viscosus

**36. A dental student is reviewing Hepatitis B serology. If a patient's labs show they are "Vaccinated," which marker will be positive?**

A. HBsAg

B. Anti-HBs

C. IgM anti-HBc

D. HBeAg

**37. Which Hepatitis virus requires the surface antigen of Hepatitis B to make its own envelope?**

A. HAV

B. HCV

/C. HDV

D. HEV

**38. A patient with HIV has a CD4 count of 180 cells/µL. According to the CDC, which stage of HIV infection are they in?**

A. Stage 1

B. Stage 2

C. Stage 3 (AIDS)

D. Recovery stage

**39. HIV is a retrovirus. What is the obligatory step in its replication where the viral DNA copy is placed into the host's cellular DNA?**

A. Reverse transcription

B. Integration

C. Budding

D. Uncoating

**40. During the synthetic phase of DNA viruses, where does replication typically occur within the host cell?**

A. Cytoplasm

B. Nucleus

C. Mitochondria

D. Cell membrane

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## Answer Key (Hidden)

1. **C** (Nagayama spots) [source:5,p23]
2. **B** (Monospot Test) [source:5,p20]
3. **C** (Trigeminal ganglion) [source:5,p4]
4. **C** (Rubeola) [source:5,p27]
5. **C** (Enteroviruses) [source:5,p34]
6. **A** (Epidermic Parotitis) [source:5,p37]
7. **B** (HHV-4/EBV) [source:5,p21]
8. **B** (Herpangina) [source:5,p33]
9. **C** (Dorsal root ganglion) [source:5,p12]
10. **C** (HPV 16) [source:5,p38]
11. **B** (Pyogenic Granuloma) [source:2,p14]
12. **B** (Prostaglandin E2) [source:2,p16]
13. **C** (Baking soda and water) [source:2,p20]
14. **B** (Second Trimester) [source:2,p21]
15. **C** (Penicillin) [source:2,p22]
16. **C** (Nitrous Oxide) [source:2,p22]
17. **C** (Prevotella intermedia) [source:6,p54]
18. **B** (1 year) [source:2,p24]
19. **D** (Hepatitis E) [source:4,p53]
20. **B** (Enamel hypoplasia) [source:2,p26]
21. **B** (Trench Mouth) [source:3,p12]
22. **C** (Treponema pallidum) [source:3,p14]
23. **C** (Gummas) [source:3,p21]
24. **A** (Bartonella henselae) [source:3,p10]
25. **B** (Tuberculosis) [source:3,p37]
26. **B** (Congenital Syphilis) [source:3,p24]
27. **A** (Scarlet Fever) [source:3,p4]
28. **C** (Tetanospasmin) [source:1,p38]
29. **B** (Clostridium difficile) [source:1,p65]
30. **C** (Lipid A/LPS) [source:1,p14]
31. **B** (Reversible attachment) [source:6,p7]
32. **D** (Red complex) [source:6,p38]
33. **B** (P. gingivalis, T. forsythia, T. denticola) [source:6,p53]
34. **B** (Gram-positive facultative to Gram-negative anaerobic) [source:6,p35]
35. **C** (Streptococcus mutans) [source:6,p43]
36. **B** (Anti-HBs) [source:4,p49]
37. **C** (HDV) [source:4,p54]
38. **C** (Stage 3) [source:4,p69]
39. **B** (Integration) [source:4,p59]

40. **B** (Nucleus) [source:4,p21]